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Solving Behavior Problems in Nonverbal Individuals with Autism

Behavior problems in nonverbal individuals with autism are often difficult to alleviate because these people cannot tell you how they feel. However, all behavior is communication. As a parent, educator, or caregiver, you have to learn to be a good detective to figure out why a nonverbal person with autism is acting out.

If a nonverbal individual who is generally calm suddenly becomes aggressive or frequent tantrums arise, first look for a hidden painful medical problem. Some of the common sources are ear infections, a bad toothache, a sinus infection, gastro-intestinal problems, acid reflux (heartburn), and constipation. You need to be observant; the individual may touch or clutch at the area of the body that hurts, avoid certain foods they previously enjoyed, or their sleep patterns may be different. Acid reflux is common in ASD and stomach pain may explain why a nonverbal individual may refuse to lie down or sit still.

Sensory overload is a second—and more frequent—cause of behavior outbursts. A tantrum in Wal-Mart or other such crowded stores is usually due to sensory overstimulation. Behavior tantrums at school may arise during or prior to times when many students are gathered together, such as recess, lunchtime, or assemblies. Stimuli that are most likely to cause problems are the flicker of fluorescent lights, perfumes/colognes and other strong smells (the cafeteria at school, the store's bakery or seafood section, the restaurant's kitchen), and high-pitched sounds such as squeaky wheels on shopping carts, store product announcements, or smoke alarms. An individual who had previously been calm or cooperative may be afraid to go into a store or a room where a microphone once had audio feedback and squealed. A place that he previously liked may now be too scary because of the association with noxious stimuli. Sounds, smells, and textures that are merely annoying to typical people may be like a dentist's drill hitting a nerve for a person with autism. I have difficulty tolerating scratchy clothes, but for some more sensitive individuals, scratchy sweaters, stiff new clothes, or double-stitched seams may cause a pain sensation. Often something as simple as changing to a new brand of socks may feel like walking on burning sandpaper.

If hidden medical problems and sensory issues are ruled out, the tantrums or outbursts may stem from a purely behavioral reason. The three major behavioral sources of tantrums, hitting and meltdowns are:

- frustration because the person cannot communicate
- the need for attention
- to escape from a task they do not want to do

Deciphering the correct motivator is important. Once the motivator has been found, a solution can be developed. Otherwise, while it is possible to extinguish an inappropriate behavior, in all likelihood an equally inappropriate behavior that meets the same need will develop. For example, ignoring the behavior is the correct response if the behavior motivator is to seek attention. However, this response would be the worst thing to do if the individual was frustrated because he could not communicate his need for help. Solutions to the behavior could include teaching the child sign language or to use an augmentative communication device. Or teaching the individual socially appropriate ways of saying “no” or expressing his desires.

Keeping a detailed diary will help you figure out the behavior motivation. One nonverbal boy screamed to get his mother to stop at McDonald’s because he had learned that it worked. However, he never screamed when his father was driving because he knew dad would not stop.

Frustration with not being able to communicate is a very common problem in nonverbal individuals with ASD; they must have a way to express their needs and wants. If a functional communication system has not been put into place with a child, his only recourse is behavior. I can remember screaming when I did not want to wear a hat. I had no other way to express my dislike for it, nor to communicate that for me, the hat was disturbing on a sensory level.

Consistency is calming; surprises produce anxiety in most individuals with ASD. They need to know what is coming up next. Sensory processing systems in some of these individuals are so disordered that touch and smell are the only two senses that provide reliable, accurate information to the person’s brain.

If their visual and auditory systems are giving them jumbled information, they may rely more on touch. This is why some nonverbal people tap things like a blind person navigating with a cane. The neurological feedback this provides is calming to their senses. It also explains some of the repetitive behaviors common to individuals with autism. The consistency provided by doing the same thing again and again—and getting the same result—alleviates some of the anxiety associated with the rest of the world being in a constant state of change.

Because of the visual processing problems experienced by many nonverbal individuals with ASD, picture schedules may work poorly with some of them. At one group home, many outbursts and tantrums were avoided by using a touch schedule instead of a picture schedule. Ten minutes before breakfast, they were given a spoon to hold and ten minutes before a shower, they were given a wash cloth. The tangible object communicated what was to happen and the ten-minute period gave their brains time to process the sensory information.

Research is also now showing very clearly that exercise will reduce anxiety and stereotypical behavior. At another group home, a program of vigorous exercise reduced behavior problems.

Some nonverbal teenagers and adults on the spectrum may need medication to alleviate stress and anxiety so that other forms of behavior modification can be used successfully. Often a combination of approaches is best. Sometimes a small dose of conventional medication combined with dietary or other biomedical treatment is more effective than either method used alone.

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